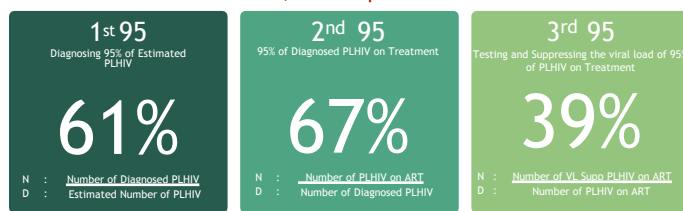




HIV & AIDS SURVEILLANCE OF THE PHILIPPINES

HIV & AIDS CONTINUUM OF CARE

95-95-95 ACCOMPLISHMENT, as of September 2024



The 95-95-95 Targets

The 95-95-95 by 2025 is the global targets set by the Joint United Nations Programme on HIV and AIDS (UNAIDS). The Philippines, as one of the States who committed to the "Political Declaration on HIV and AIDS: Ending Inequalities and Getting on Track to End AIDS by 2030" adopted during the General Assembly in June 2021, integrated these high-level targets in the 7th AIDS Medium Term Plan - 2023 to 2028 Philippines: Fast Tracking to 2030. It aims that by 2030, 95% of people living with HIV know their HIV status or are diagnosed, 95% of PLHIV who know their status are receiving treatment (ART), and 95% of PLHIV on ART have a suppressed viral load so their immune system remains strong, and the likelihood of their infection being passed on is greatly reduced (Undetectable=Untransmissible).

The Philippine People Living with HIV (PLHIV) Estimates

The Philippines has been using the national PLHIV estimates to determine the state and trend of the epidemic in the country, to aid programmatic response and develop strategic plans, and to monitor progress towards the 95-95-95 targets. Annually, the National HIV/AIDS & STI Surveillance and Strategic Information Unit of the Department of Health-Epidemiology Bureau leads the process of developing the PLHIV estimates, which was modeled through the AIDS Epidemic Model (AEM) and Spectrum. The latest PLHIV estimates were updated in May 2023 with analyzed and triangulated data from the 2022 HIV/AIDS & ART Registry of the Philippines (HARP), 2018 Integrated HIV Behavioral and Serologic Surveillance (IHBS), 2019 and 2020 Online Survey among Males having Sex with Males and Transgender Women (MSM & TGW), 2019 Facility-based Survey, 2022 Laboratory and Blood Bank Surveillance (LaBBS), 2020 Population Census, and other program data. Further, the development of PLHIV estimates underwent a comprehensive consultation, validation, and vetting process with technical experts from EastWest Center, UNAIDS, WHO, and key national, regional, and local program implementers and stakeholders. Previously released estimates in May 2022 were based on the IHBS 2018, HARP December 2020, and Population Census 2015.

Diagnosed PLHIV

The total number of diagnosed or laboratory-confirmed HIV cases reported in the HIV/AIDS Registry who are currently alive or not yet reported to have died.

PLHIV on ART

A PLHIV who is currently on ART defined as visited the facility for an ARV refill or accessed ARV refill, and has not run-out of pill for 30 days.

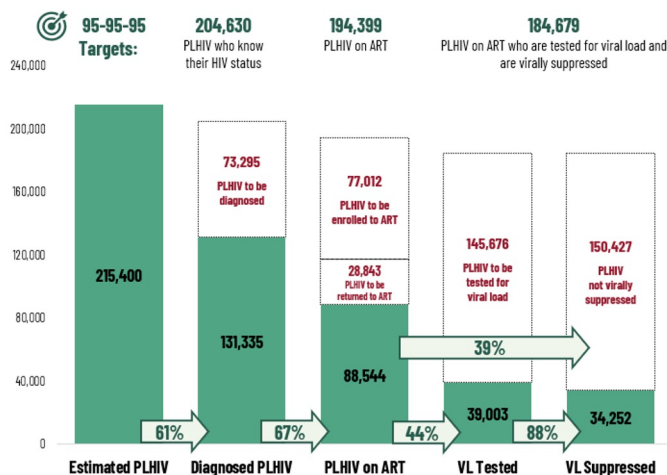
Virally Suppressed PLHIV

PLHIV on ART who have viral load of ≤ 50 copies/mL. Viral load refers to the amount of the Human Immunodeficiency virus (HIV) present in an infected person's blood.

The latest Philippine HIV estimates show that by the end of 2024, there will be 215,400 estimated People Living with HIV (PLHIV) in the country.

Of the estimated PLHIV, 131,335 (61%) cases have been diagnosed or laboratory-confirmed and currently living or not reported to have died, as of September 2024. Further, 88,544 (67%) PLHIV are currently on life-saving Antiretroviral Therapy (ART), of which, 39,003 (44%) PLHIV have been tested for viral load (VL) in the past 12 months. Among those tested for VL, 34,252 (88%) are virally suppressed. However, only 39% were virally suppressed among PLHIV on ART [Figure 1].

Figure 1. National Care Cascade as of September 2024

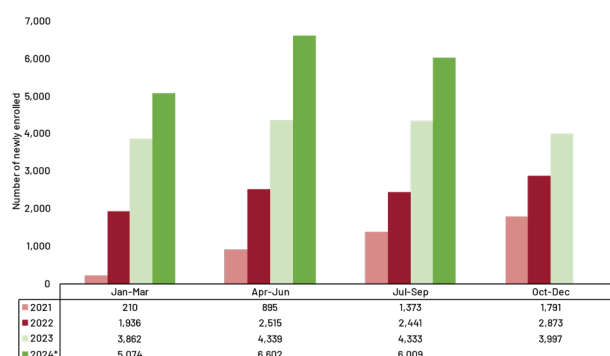


See Annex A: HIV Care Cascade per Region, Age Group and Key Population

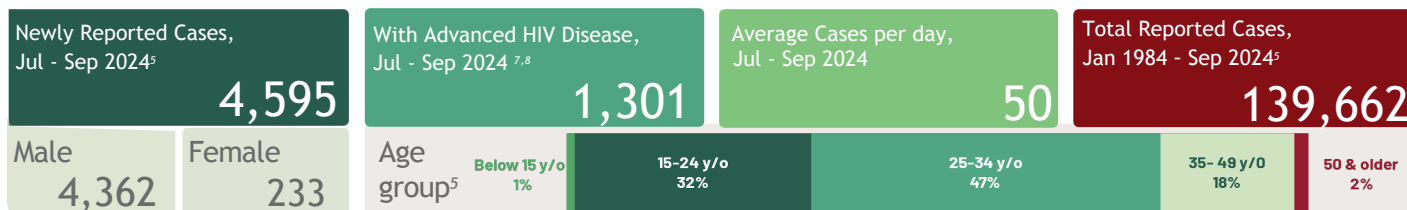
PREVENTION

In July to September 2024, there were 6,009 clients newly enrolled to Pre-Exposure Prophylaxis (PrEP), which is a 39% increase in new enrollees compared to the same period in 2023. Of the enrollees in the 3rd quarter, 86 (1%) were less than 18 years old at the time of enrollment, 2,431 (40%) were 18-24 years old, 2,683 (45%) were 25-34 years old, 803 (13%) were 35 years old and above^{1,2}. More than half (55%, 3,285) of the newly enrolled to PrEP were from the National Capital Region (NCR).

Since the implementation of PrEP in March 2021, a total of 48,250 clients have been enrolled. Of these, 46,848 (97%) were male, and 65% (31,308) were 25 years old or older. The majority of clients ever enrolled in PrEP (88%, 42,490) were registered at facilities in the NCR, CALABARZON (4A), and Central Luzon (3) regions³. As of September 2024, among the total enrolled, over half (59%, 28,339) returned for a PrEP refill in 2024, of which 48% or 13,527 were 2024 new enrollees. Of the 19,911 non-returnees, 3% (679) tested positive for HIV.

Figure 2. Quarterly PrEP Enrollment, as of September 2024 (n=48,250)⁴

DIAGNOSIS



In July to September 2024, there were 4,595 confirmed HIV-positive individuals reported to the One HIV/AIDS & STI Information System (OHASIS), of which, 1,301 (28%) had an advanced HIV infection^{5,6} at the time of diagnosis. Compared to last year's 3rd quarter average of 51 cases per day, there has been no significant difference with this year's daily average of 50 cases.

Of the newly reported confirmed HIV-cases this period, 4,362 (95%) were males while 233 (5%) were females. The age of the newly reported cases ranged from 1 to 73 years old (median: 27 years). By age group, 24 (<1%) were less than 15 years old at the time of diagnosis, 1,472 (32%) were 15-24 years old, 2,179 (47%) were 25-34 years old, 808 (18%) were 35-49 years old, and 112 (2%) were 50 years and older⁴. Moreover, 3,210 (70%) were cisgender, 135 (3%) identified themselves as transgender women, 33 (<1%) identified as others, 30 (1%) as neither man nor woman, eight (<1%) as transgender man and 1,179 (26%) had no data on gender identity⁸. Of the newly reported cases, 3,204 (70%) were confirmed in Certified Rapid HIV Diagnostic Algorithm (rHIVda) Confirming Laboratories (CrCLs) while 1,391 (30%) were confirmed through the National Reference Laboratory-San Lazaro Hospital/STD AIDS Cooperative Central Laboratory (NRL-SLH/SACCL).

1. Age at the time of enrollment to PrEP; 6 has no data on age among 3rd quarter of 2024 enrollees

2. Percentages were rounded off to the nearest whole number - sum may not be equal to 100% due to rounding of figures

3. Based on the region of PrEP facility; 4 had no data on PrEP facility; 96 were from Overseas

4. Difference in totals from previous quarters were due to late reporting from the sites

5. Age at diagnosis for 3rd quarter; Reported diagnosed HIV cases, including deaths

6. Advanced HIV Disease (AHD) definition is based on clinical criteria of WHO staging 3 and 4 while immunologic criterion is based on baseline CD4 results (<200 cells/mm³)

7. 1,928 cases had non-advanced HIV infection and 741 had no data on immunologic/clinical criteria at the time of diagnosis

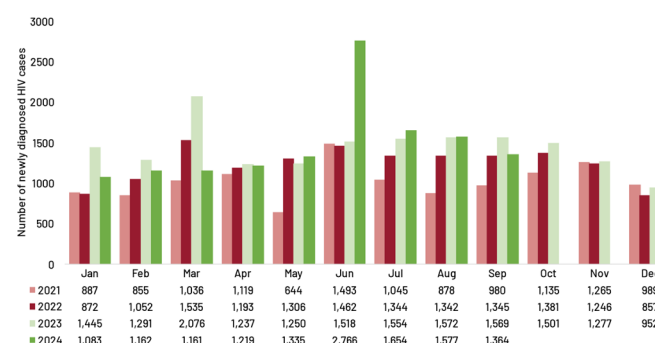
8. Gender identity is based on sex at birth and self identity reported at the time of diagnosis. Those with unknown gender identity either had unspecified or no data on self identity and/or sex at birth.

Cumulatively, 139,662 confirmed HIV cases have been reported to the HIV/AIDS and ART Registry of the Philippines since the first reported HIV case in the Philippines in 1984 [Figure 3].

Since 2021, the number of newly diagnosed HIV cases reported monthly has been increasing [Figure 3]. The average monthly cases were 1,027 in 2021, increased by 21% or 1,245 in 2022, and then rose by an additional 15% or 1,437 in 2023. A slight increase in the average diagnosis from January to September 2024 was observed at 1,480 monthly.

Moreover, the number of reporting Certified rHIVda Confirming Laboratories (CrCLs) in OHASIS increased from 26 facilities in 2021 to 74 facilities as of September 2024.

Figure 3. Number of monthly newly diagnosed HIV cases, Jan 2021-Sep 2024



Geographic Distribution

From July to September 2024, the regions with highest reporting of newly diagnosed cases were NCR, CALABARZON (4A), Central Luzon (3), Central Visayas (7), Western Visayas (6), and Davao region (11) accounting for 72% of the total cases while 1,220 cases (26%) were reported from other regions [Figure 4]⁹. Between January 1919 and June 2024, these same regions contributed to 77% or 60,587 of the total reported cases. The remaining 17,899 cases (23%) were distributed across other regions [Table 1]¹⁰.

Similarly, from January 1984 to September 2024, these regions have consistently reported the highest number of cases, with a total of 111,288 cases, representing 80% of all reported cases [Table 1]. During this period, 27,194 cases (19%) were reported from other regions within the country, eleven cases (<1%) were reported from overseas, and 1,169 cases (1%) had no data on their region of residence.

Figure 4. Distribution of newly diagnosed HIV cases by region of residence⁹, Jul- Sep 2024 (n= 4,595)

Region	Number of Cases	%
NCR	1,017	22%
4A	784	17%
3	554	12%
7	370	8%
6	336	7%
11	298	6%
5	213	5%
1	181	4%
10	149	3%
4B	133	3%
2	117	3%
8	110	2%
12	99	2%
CARAGA	90	2%
9	61	1%
CAR	45	1%
BARMM	22	<1%



Table 1. Number of diagnosed HIV cases, by region of residence, Jan 1984 - Sep 2024¹⁰

Region	January 2024 – September 2024 (n=13,321) ¹⁰		January 2019 - September 2024 (n=78,575) ¹⁰		January 1984 - September 2024 (N=139,662) ¹⁰	
NCR	3,145	24%	21,634	28%	45,660	33%
4A	2,425	18%	14,119	18%	23,243	17%
3	1,485	11%	9,042	12%	14,682	11%
7	1,011	8%	5,835	7%	11,301	8%
6	1,004	8%	5,615	7%	8,607	6%
11	731	5%	4,342	6%	7,795	6%
1	508	4%	2,514	3%	3,838	3%
12	384	3%	2,381	3%	3,794	3%
10	421	3%	2,426	3%	3,707	3%
5	449	3%	2,056	3%	3,063	2%
8	336	3%	1,629	2%	2,401	2%
2	325	2%	1,624	2%	2,360	2%
4B	354	3%	1,610	2%	2,295	2%
9	196	1%	1,400	2%	2,214	2%
CARAGA	275	2%	1,204	2%	1,808	1%
CAR	140	1%	730	1%	1,218	1%
BARMM	59	<1%	325	<1%	496	<1%

Sex and Age

Majority of the total reported cases (131,776, 94%) were males and 7,876 (6%) were females [Figure 5]¹¹. By age group, 471 (<1%) were below 15 years old, 41,219 (30%) were among the youth aged 15-24 years old, half (69,808, 50%) were 25-34 years old, 24,478 (18%) were 35-49 years old, and 3,610 (3%) were 50 years and older¹². The age of diagnosed cases ranged from <1 to 81 years old (median: 28 years).

Since 2012, the proportion of males among the newly diagnosed cases has consistently been at least 95%. Moreover, diagnosed HIV cases are getting younger with the predominant age group shifting from among 35-49 years old in 2002 to 2005, to 25-34 years old starting 2006 [Figure 6]. Among age groups, the most significant increase in the proportion of cases over the past five years (2019-2023) compared to the period from 1984-2018 was observed in individuals below 15 years old. Specifically, there were 179 cases in this age group from 1984 to 2018, which rose to 292 cases from 2019 to September 2024.

Figure 5. Proportion of diagnosed HIV cases, by sex, Jan 1984 – September 2024¹¹

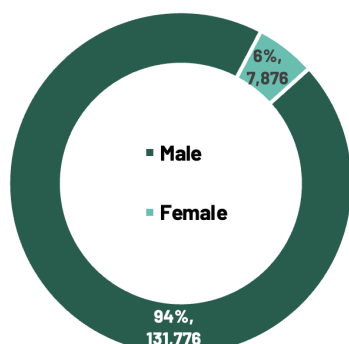
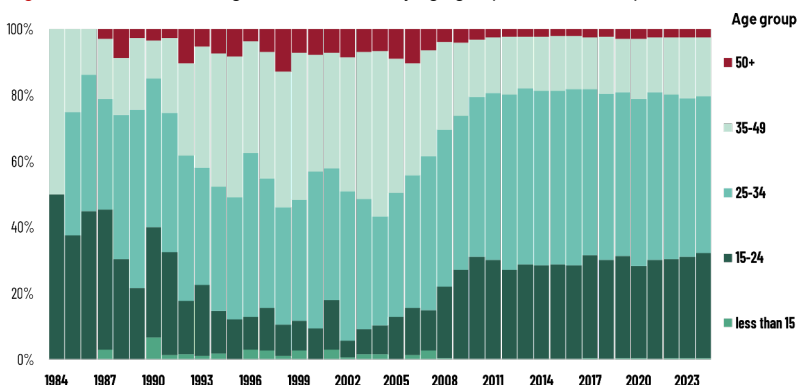


Figure 6. Distribution of diagnosed HIV cases, by age group, Jan 1984 – September 2024¹²



9. 13 (<1%) had no data on residence cases reported on July-Sep 2024 while 3 (<1%) were from overseas;

10. From Jan-Sep 2024, 65 had no data on residence while 8 were from overseas; From Jan 2019- Sep 2024, 78 (<1%) had no data on residence while 11 (<1%) were from overseas; Since 1984, 1,169 cases (1%) had no data on residence and 11 (<1%)

11. No data on sex for 10 cases

12. No data on age for 76 cases

Mode of Transmission (MOT)

In the third quarter of this year, 4,424 (96%) newly reported cases had acquired HIV through sexual contact - 3,263 through male-male sex, 623 male - female¹³, and 538 male - female sex. Meanwhile, 17 (<1%) reported sharing of infected needles, 13 (<1%) through mother-to-child transmission, and 141 had no data on mode of transmission at the time of diagnosis [Table 3].

Table 3. Number of diagnosed HIV cases, by mode of transmission and sex, Jan 1984 - September 2024^{14, 15}

Mode of Transmission	January 2024 – September 2024 (n=13,321) ¹⁵		January 2019 – September 2024 (n=78,575) ¹⁵		January 1984 – September 2024 (n=139,662) ¹⁵	
	M	F	M	F	M	F
	(12,564)	(757)	(74,556)	(4,019)	(131,776)	(7,876)
Sexual Contact	12,060	695	72,521	3,770	127,136	7,333
Male-male	9,291	-	51,544	-	82,665	-
Both males & females ¹³	1,779	-	15,428	-	32,592	-
Male-female	990	695	5,549	3,770	11,879	7,333
Sharing of infected needles	53	3	522	37	2,474	157
Mother-to-child	20	18	102	106	191	183
Blood /blood products	-	-	-	-	5	14
Needlestick injury	-	-	-	-	2	1
No data	431	41	1,411	106	1,968	188

Sexual contact has consistently been the leading mode of HIV transmission among newly diagnosed cases over the years [Figure 7]. From January 1984 to September 2024, of the 139,662 reported cases, 134,496 (96%) were acquired through sexual contact. This includes 82,665 cases from male-male sex, 32,592 from male-male/female sex, and 19,212 from male-female sex [Table 3].

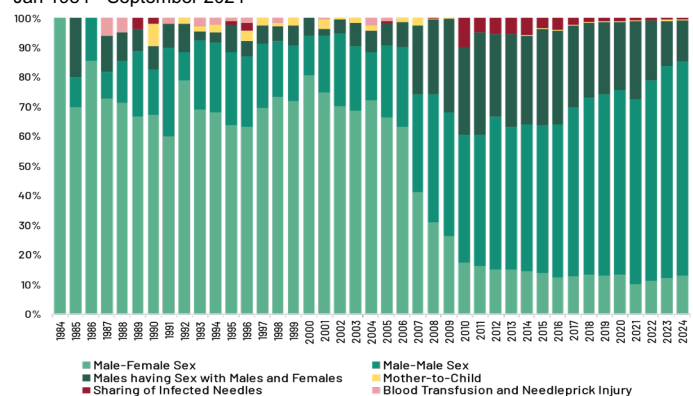
In contrast, there has been a notable increase in HIV cases resulting from mother-to-child transmission. Out of a total of 374 cases, more than half (56%, 208) were reported between January 2019 and September 2024 [Table 3].

Additionally, sharing of infected needles has consistently accounted for 2% (2,631) of the total cases. Transmission through blood/blood products and needlestick injuries has been reported in 22 cases (<1%), with no new cases reported since 2011. Furthermore, 2,156 cases (1%) have an unknown mode of transmission.

Among diagnosed male cases, 115,257 (87%) acquired HIV through sex with another male, 11,879 (9%) through sex with a female, 2,474 (2%) through sharing of infected needles, and 191 (<1%) through mother-to-child transmission. Similarly, among diagnosed females, the predominant mode of transmission was sexual contact, with 93% (7,333) acquiring HIV through sex with a male. Additionally, 183 cases (2%) were attributed to mother-to-child transmission, and 157 cases (2%) were due to sharing infected needles [Table 3].

Modes of transmission (MOT) show regional variations. For instance, 48% of diagnosed males who have sex with males were from NCR; over half of those who acquired HIV through mother-to-child transmission were from NCR, Region 4A, and Region 3 (59%); and almost all (99%) who acquired HIV through sharing of infected needles among people who inject drugs were from Region 7.

Figure 7. Distribution of diagnosed HIV cases, by mode of transmission, Jan 1984 - September 2024¹⁵

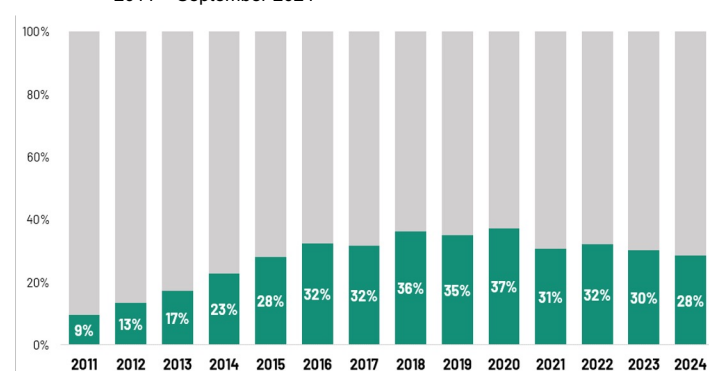


Advanced HIV Disease (AHD)

Reporting of Advanced HIV Disease (AHD)¹⁶ cases only started in 2011. Among the total reported cases, 40,478 (30%) were diagnosed with Advanced HIV disease (AHD). Notably, data on immunologic and clinical criteria at the time of diagnosis were unavailable for the remaining 98,273 (70%) cases.

From 2011 to 2020, there was a notable increase in the proportion of cases with AHD, rising from 9% in 2011, with a median baseline CD4 count of 128 cells/mm³, to 37% in 2020 [Figure 8], with a median baseline CD4 count of 198 cells/mm³. Over the past 5 years, there was a 17% increase in AHD cases with a median CD4 count of 227 cells/mm³. This trend experienced a slight decline to 31% in 2021, 32% in 2022, 30% in 2023, and further decreased to 28% from January to September of 2024. In comparison to the third quarter of 2023, the median baseline CD4 count saw a slight increase from 226 cells/mm³ in 2023 to 235 cells/mm³ in 2024.

Figure 8. Proportion of newly diagnosed HIV cases with advanced HIV disease¹⁶, 2011 – September 2024



TREATMENT

Antiretroviral Therapy (ART)

Newly Enrolled to ART,
Jul- Sep 2024¹⁷

4,492

Median Baseline CD4
at enrollment (in
cells/mm³)¹⁸

217

PLHIV on ART as of Sep 2024

88,544

Current age (in years)¹⁹

Age Range 1 - 82
Median Age 32

Sex assigned at
birth

Male 85,194
Female 3,350

In July to September 2024, there were 4,492 people with HIV who were enrolled to treatment, of which, 4,386 (98%) were on the first line regimen, eleven were on second line regimen, and 95 were on other line of regimen. Among them, 32 (1%) were less than 15 years old, 1,409 (31%) were 15-24 years old, 2,094 (47%) were 25-34 years old, 835 (19%) were 35-49 years old, and 122 (3%) were 50 years and older. The median CD4¹⁸ of these patients upon enrollment was at 217 cells/mm³.

13. Among males only

14. Sex at birth: M=Male, F=Female

15. No data on sex and MOT for 10 cases

16. Classification of diagnosed cases with advanced clinical manifestations based on immunologic and clinical criteria has been newly implemented in 2022. Previously advanced HIV cases were identified based solely on available clinical criteria.

17. Started on ART are those enrolled from July to September 2024 regardless of diagnosis date

18. No data on baseline CD4 count for 2,100 cases newly enrolled to ART from July to September 2024

19. Current age as of the reporting period



Among the 117,203 people living with HIV (PLHIV) who have ever been enrolled in antiretroviral therapy (ART) since 2002, a total of 88,544 individuals aged 1 to 82 years old (median age: 32 years) were alive on ART as of September 2024. Of these, 86,609 were on a first-line regimen, 1,021 were on a second-line regimen, and 913 were on other lines of regimen.

As of September 2024, 23,314 individuals (20%) who were previously on ART were no longer receiving treatment. This group includes 23,296 individuals who were lost to follow-up (LTFU), five who refused to continue ART for various reasons, and 13 who reported migrating overseas [Table 4].

Sixty-three percent of the PLHIV on ART are concentrated in the Greater Manila Area (GMM), which includes NCR, CALABARZON (4A), and Central Luzon (3). Conversely, the highest rates of lost to follow-up are observed in NCR, followed by Central Visayas (Region 7) and CALABARZON (4A). Together, these three regions contribute to 65% of the total number of PLHIV not on treatment in the country.

Viral Load (VL) Testing and Suppression

Among the PLHIV on ART as of September 2024, a total of 84,052 individuals had been enrolled in ART for at least 3 months and were tagged as eligible for viral load testing. Of these eligible individuals, 39,003 PLHIV (46%) underwent viral load testing within the past 12 months. Specifically, 6,959 (18%) were tested between July and September 2024, 9,825 (25%) between April and June 2024, 13,201 (34%) between January and March 2024, and 9,018 (23%) between October and December 2023.

Further, among the 39,003 PLHIV on ART who were tested in the past 12 months as of September 2024, 34,252 (88%) were virally suppressed²⁵ while 4,751 (12%) were not virally suppressed. Moreover, there has been a notable 178% increase in coverage over the past 5 years (2019-2023). On the other hand, viral suppression rates have ranged from 82-88% since 2019 while viral load testing coverage has consistently remained below 50%.

Regionally, Western Visayas (Region 6), Central Luzon (Region 3), and NCR have reached viral load testing coverage exceeding 50%, with suppression rates ranging from 84-90%. In contrast, other regions have reported coverage below 50%, with suppression rates varying from 72-93%, the lowest being in Region 9[Table 5].

Figure 9. Viral Load Testing and Suppression among PLHIV on ART, 2019 - September 2024^{25,26}

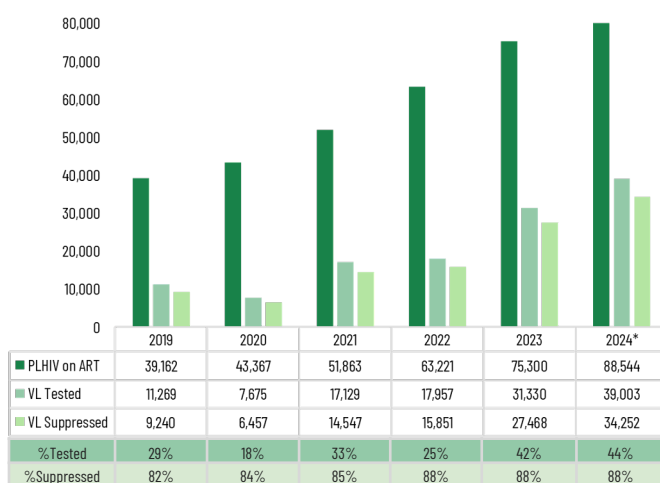


Table 4. Number of PLHIV by treatment outcome and region, as of September 2024

Region of Treatment Facility ²⁰	Treatment Outcome			
	Alive on ART ^{20,21} (n= 88,544)	Lost to Follow-up ²² (n= 23,296)	Trans out (Overseas) ²³ (n= 13)	Stopped ²⁴ (n= 5)
NCR	38,755	10,606	-	-
4A	9,624	2,094	1	-
3	7,704	1,931	7	1
7	6,907	2,479	-	-
11	5,632	1,467	-	-
6	5,610	840	-	-
12	2,844	632	-	-
10	2,212	539	-	-
1	1,616	373	-	-
5	1,421	477	-	-
2	1,378	206	1	-
8	1,069	345	-	-
4B	1,027	340	-	-
9	987	374	4	-
CAR	974	186	-	-
CARAGA	704	381	-	4
BARM	79	26	-	-

Table 5. Viral load testing and Suppression²⁵ among PLHIV on ART by region, as of September 2024

Region of Treatment Facility ²⁰	Viral Load Status among PLHIV on ART per region				
	Alive on ART (n= 88,544) ^{20,21}	Tested for VL (n= 39,003)	% Tested for VL	VL Suppressed (n= 34,252)	% Suppressed
NCR	38,755	17,694	46%	15,957	90%
4A	9,624	4,349	45%	3,649	84%
3	7,704	4,024	52%	3,421	85%
7	6,907	2,669	39%	2,358	88%
6	5,632	2,404	43%	2,164	90%
11	5,610	3,278	58%	2,901	88%
12	2,844	1,032	36%	846	82%
10	2,212	242	11%	156	64%
1	1,616	510	32%	392	77%
5	1,421	935	66%	785	84%
2	1,378	710	52%	631	89%
8	1,069	349	33%	295	85%
CAR	1,027	443	43%	387	87%
4B	987	26	3%	20	77%
9	974	260	27%	237	91%
CARAGA	704	77	11%	53	69%
BARM	79	1	1%	0	0%

MORTALITY

Newly reported deaths
Jul-Sep 2024

459

Total reported deaths
Jan 1984 - Sep 2024²⁷

8,327

From January 2019 to September 2024, there have been 5,350 deaths reported among diagnosed HIV cases in the Philippines, with more than 500 new deaths reported each year since 2018.

Since January 1984, a total of 8,327 deaths have been reported, of which, 3,918 (47%) had an advanced HIV disease at the time of diagnosis²⁸. Among age groups, the largest proportion of reported deaths were among the 25-34 years old accounting for 4,171 (48%) of total deaths followed by 35-49 years old with 2,205 (26%), 15-24 years old with 1,701 (20%), 50 years old and older with 450 (5%), and <15 years old with 73 (1%). Eight (<1%) of the reported deaths had no reported age at the time of death.

20. Current treatment facility where PLHIV last visited for ARV refill; 1 had no region of treatment facility

21. PLHIV is alive on ART if he/she visits the treatment facility for ARV refill within 30 days from expected day of last (run-out) pill

22. PLHIV is lost to follow-up if he/she did not visit the treatment facility for ARV refill within 30 days from expected day of last (run-out) pill

23. Clients who reported to have migrated or transferred to another country

24. Clients who stopped due to refusal to treatment

25. Viral Suppression is defined as having less than 50 copies of HIV per milliliter of blood, based on DOH AO 2022-0024

26. PLHIV currently alive on ART with at least 1 visit and screened within the reporting period

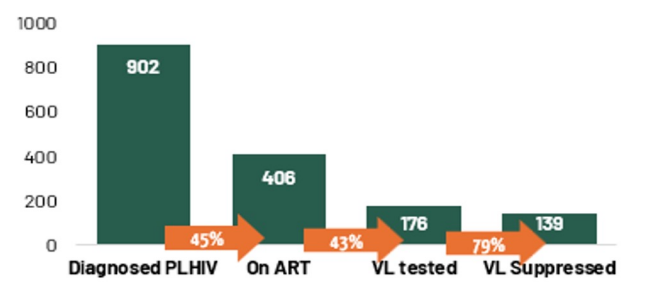
27. Reported deaths due to any cause and not limited to AIDS-related causes. Based on reported date, and actual date of death may not necessarily fall in this reporting period

28. 4,409 (53%) had no data on immunologic/clinical criteria at the time of diagnosis.

OTHER VULNERABLE POPULATIONS

Pregnant Women with HIV

Figure 10. Diagnosis and Treatment coverage among PLHIV Diagnosed during pregnancy



From July to September 2024, there were 32 HIV positive women aged 17 to 46 years old (median: 22 years) who were pregnant at the time of diagnosis. This was an 18% decrease compared to the same reporting period last year.

The reporting of pregnancy status at the time of diagnosis was integrated into HARP in 2011, and since then, a total of 935 diagnosed women were reported pregnant at the time of diagnosis. Moreover, over the past 5 years, there has been a 11% increase in reported cases.

Among the pregnant women at the time of diagnosis, 902 (96%) were currently alive. Of these, 729 (81%) were initiated to ART however, only 406 (43%) among them were retained on ART. Of those who were on treatment, only 176 (43%) were tested for viral load with 79% (139) viral load suppression [Figure 10].

Transgender Women (TGW)

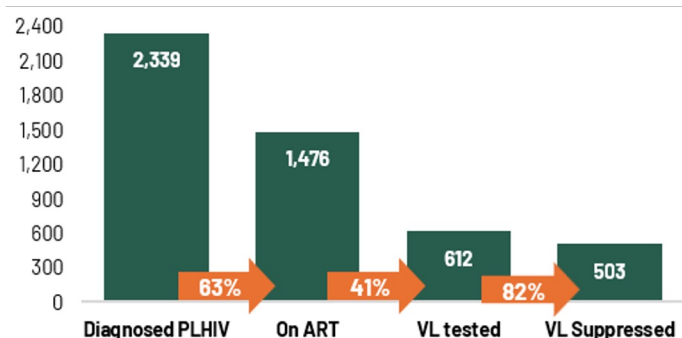
from July to September 2024 there were 135 newly reported cases who identified as transgender women²⁹ where 40 (30%) were 15 - 24 years old, 68 (50%) were 25 - 34 years old, 24 (18%) were 35-49 years old, and three (2%) were 50 years and older. The age of diagnosis ranged from 18 to 65 years old (median: 22 years).

Of the 2,491 TGW diagnosed with HIV from January 2018³⁰ to September 2024, almost all (2,462, 99%) acquired HIV through sexual contact, five (<1%) through sharing of infected needles, and 22 (1%) had no data on MOT. By age group, 703 (28%) were 15-24 years old at the time of diagnosis, half (1,242, 50%) were 25-34 years old, 487 (20%) were 35-49 years old, and 58 (2%) were 50 years and older, and one had no data on age. The age of diagnosis ranged from 15 to 65 years old (median: 22 years).

Among the diagnosed cases of TGW, 2,339 (94%) were currently alive. Of these, 2,031 (87%) were initiated to ART however, only 1,476 (63%) among diagnosed TGW living with HIV were retained on ART. Of those who were on treatment, only 612 (41%) were tested for viral load with 82% (503) viral load suppression [Figure 11].

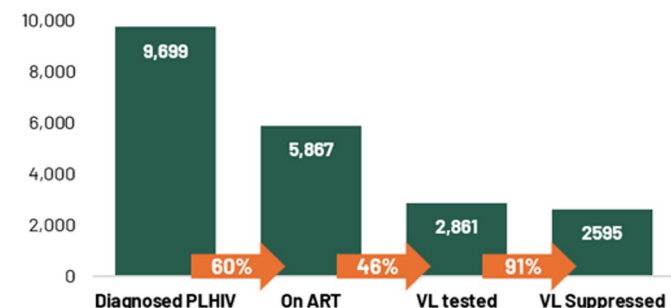
Of those who were on treatment, only 556 (41%) were tested for viral load with 82% (457) viral load suppression [Figure 11].

Figure 11. Diagnosis and Treatment coverage among TGW living with HIV



Migrant Workers

Figure 12. Diagnosis and Treatment coverage among Migrant Workers living with HIV



Among the diagnosed cases of Migrant workers, 9,861 (94%) were currently alive. Of these, 7,647 (78%) were initiated to ART however, only 6,068 (62%) among diagnosed living with HIV were retained on ART. Of those who were on treatment, only 2,904 (48%) were tested for viral load with 90% (2,626) viral load suppression [Figure 12].

From July to September 2024, 181 (4%) of reported cases were Filipinos aged 20 to 67 (median: 43) who had worked overseas in the past five years. Of these, 160 (88%) were male and 21 (12%) were female. Most (175, 97%) acquired HIV through sexual contact: 104 (57%) through male-male sex, 51 (14%) through sex with both males and females, and 58 (28%) through male-female sex; 6 (3%) had no data on transmission. There was a 16% decline in HIV diagnoses among migrant workers compared to the same period last year, and a 14% decrease over the past five years.

Since 1984, 10,460 (8%) of diagnosed cases have been migrant workers. Of these, 10,262 (98%) acquired HIV through sexual contact, 19 (<1%) through needle sharing, 9 (<1%) through exposure to blood, 3 (<1%) through needlestick injury, and 167 (2%) had no data on transmission.

People Engaging in Transactional Sex

In July to September 2024, 607 (13%) of the newly diagnosed engaged in transactional sex within the past 12 months. Majority (591, 97%) were males and 16 (3%) were females, their age ranged from 14 to 62 years old (median: 25 years). Of the male cases, 230 (39%) reported accepting payment for sex only, 264 (45%) reported paying for sex only, and 97 (16%) engaged in both. On the other hand, among female cases, 8 (50%) accepted payment for sex, three (19%) reported paying for sex only, and five (31%) engaged in both. Sixty-one percent or 9,505 of the total cases who had history of transactional sex were diagnosed from 2019- September 2024, of which almost half (49%) of them paid for sex [Table 6].

Since the reporting of transactional sex began in December 2012, a total of 15,480 cases have been reported to HARP³¹. The majority, 15,033 (97%), were males, while 447 (3%) were females. Among them, 5,147 (33%) accepted payment for sex, 7,889 (51%) paid for sex, and 2,444 (16%) engaged in both.

Among the diagnosed cases who had history of transactional sex, 14,410 (93%) were currently alive. Of these, 12,684 (88%) were initiated to ART however, only 9,327 (74%) among of them were retained on ART. Of those who were on treatment, only 4,096 (44%) were tested for viral load with 87% (3,544) viral load suppression.

Table 6. Diagnosed HIV cases who engaged in transactional sex, by sex and age, Dec 2012 – Sep 2024 (n= 14,875)^{32,33}

Type of Transactional Sex	January 2024 - September 2024 (n=1,666)	January 2019- September 2024 (n=9,505)	December 2012- September 2024 (N=15,480)
Accepted	642 (39%)	3,268 (34%)	5,147 (33%)
Male	615	3,137	4,870
Female	27	131	277
Age range (Median)	14-62 (25)	14-62 (26)	12-68 (26)
Paid for sex only	738 (44%)	4,695 (49%)	7,889 (51%)
Male	733	4,669	7,842
Female	5	26	47
Age range (Median)	15-71 (33)	13-80 (32)	13-80 (32)
Engaged in both	286 (17%)	1,542 (16%)	2,444 (16%)
Male	275	1,498	2,321
Female	11	44	123
Age range (Median)	14-62 (29)	14-73 (29)	14-73 (29)

29. Transgender woman tagging is based on the reported gender identity of clients which is female

30. Reporting of gender-identity in HARP started in 2018

31. People engaging in transactional sex includes all individuals who reported having either accepted payment, paid for sex, or done both in the form of money or in kind in the past 12 months. This also encompasses other key populations with similar experiences. Reporting of transactional sex was included in the HARP starting December 2012.

32. Transactional sex within the past 12 months at the time of diagnosis

33. Cumulative number of cases reported regardless when the person engaged in transactional sex. Reporting of specific time period when the person last engaged in transactional sex started only in 2017 (Form version 2017)



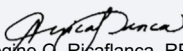
HIV & AIDS Surveillance of the Philippines - Editorial Team


Ruela P. Haynes
Surveillance Officer

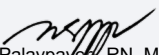

Niel Gabriel Saplagio
Surveillance Officer


Jessabel Sabejon
Surveillance Officer

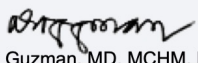

Jayne Yang
Surveillance Officer


Regine O. Ricaflanca, RPh
M&E Strategic Information
Specialist


John Benedict R. Palo
Database Specialist


Noel S. Palaypayon, RN, MGM-ESP
Manager, NHSSS


Alfonso Miguel Regala, MD, MBA
Division Chief, Monitoring and Evaluation Division (MED)


Alethea R. De Guzman, MD, MCHM, PHSAE
Director IV, Epidemiology Bureau

HIV & AIDS Surveillance of the Philippines

The HIV & AIDS Surveillance of the Philippines (HASP) is the official record of total number of diagnoses (laboratory-confirmed), ART outcome status and deaths among people with HIV in the Philippines. All individuals in the registry are confirmed by the San Lazaro Hospital STD/AIDS Cooperative Central Laboratory (SACCL) which is the HIV/AIDS National Reference Laboratory (NRL) and DOH Certified Rapid HIV Diagnostic Algorithm - rHIVda Confirmatory Laboratories (CrCLs). Confirmed HIV positive individuals were reported to the DOH-Epidemiology Bureau (EB) and recorded to OHASIS. ART figures are counts of HIV positive adult and pediatric patients currently enrolled and accessing Antiretroviral (ARV) medication during the reporting period in 230 treatment hubs and primary HIV care treatment facilities that had reported in EB. This report did not include patients who have previously taken ARV but have died, left the country, have been lost to follow-up and/or opted not to take ARV. Lost to follow-up is considered once a person have failed to visit a treatment facility 1 month after the expected date of ARV refill. HASP is a passive surveillance system. Except for HIV confirmation by the NRL & CrCLs, all other data submitted to the HASP are secondary and cannot be verified. Hence, it cannot determine if an individual's reported place of residence was where the person got infected, or where the person lived after being infected, or where the person is presently living. This limitation has major implications on data interpretation. Readers are advised to interpret the data with caution and consider other sources of information before arriving at conclusions.



National HIV/AIDS & STI Surveillance and Strategic Information Unit (NHSSS)



Department of Health 2/F Bldg. 19, San Lazaro Compound, Sta. Cruz, Manila



1003 Philippines



+632 8651-7800 local 2952



nhsss@doh.gov.ph



ebreports@doh.gov.ph



<https://doh.gov.ph/>



@hivepicenter



For further details or data requests not covered in this report, please send us your inquiries.

Access a list of facilities offering HIV services at:

tinyurl.com/HIVFacilities
or by scanning the QR Code





HIV Care Cascade

Care Cascade by Region

REGION	ESTIMATED PLHIV	DIAGNOSED PLHIV	1st 95) (Dx PLHIV/ Est. PLHIV)	ON ART	2nd 95 (On ART/ Dx PLHIV)	VL TESTED	VL TESTING COVERAGE (VL Tested/On ART)	VL SUPPRESSED	VL SUPPRESSION AMONG TESTED	3rd 95 (VL Suppressed/ On ART)
NCR	60,800	43,528	72%	27,743	64%	12,832	46%	11,479	89%	41%
4A	38,400	22,142	58%	14,738	67%	6,876	47%	5,975	87%	41%
3	25,300	13,562	54%	9,243	68%	4,680	51%	4,048	86%	44%
7	17,900	10,610	59%	6,151	58%	2,474	40%	2,177	88%	35%
6	15,000	7,587	51%	5,763	76%	3,270	57%	2,891	88%	50%
11	13,000	7,415	57%	5,026	68%	2,120	42%	1,905	90%	38%
1	6,600	3,608	55%	2,356	65%	840	36%	705	84%	30%
12	6,600	3,609	55%	2,553	71%	1049	41%	914	87%	36%
10	6,000	3,442	57%	2,417	70%	399	17%	304	76%	13%
5	5,100	2,903	57%	1,933	67%	1119	58%	960	86%	50%
2	4,000	2,227	56%	1,643	74%	811	49%	727	90%	44%
8	3,800	2,241	59%	1,486	66%	581	39%	506	87%	34%
9	3,700	2,077	56%	1,249	60%	184	15%	160	87%	13%
4B	3,600	2,136	59%	1,404	66%	624	44%	552	88%	39%
CARAGA	2,800	1,681	60%	983	58%	209	21%	175	84%	18%
CAR	2,100	1,152	55%	821	71%	282	34%	247	88%	30%
BARMIM	800	465	58%	274	59%	97	35%	76	78%	28%

Footnote: Regional cascade is based on the residence of the HIV-positive individual at the time of diagnosis.; Dx = Diagnosed

Care Cascade by Age Group

REGION	ESTIMATED PLHIV	DIAGNOSED PLHIV	1st 95) (Dx PLHIV/ Est. PLHIV)	ON ART	2nd 95 (On ART/ Dx PLHIV)	VL TESTED	VL TESTING COVERAGE (VL Tested/On ART)	VL SUPPRESSED	VL SUPPRESSION AMONG TESTED	3rd 95 (VL Suppressed/ On ART)
CHILDREN (<10)	1,300	236	18%	199	84%	80	40%	44	55%	22%
ADOLESCENTS (10-19)	11,300	1,643	15%	1,129	69%	324	29%	239	74%	21%
YOUTH (15-24)	50,700	12,146	24%	8,965	74%	3,179	35%	2,642	83%	29%
ADULTS (25+)	168,400	117,238	70%	78,251	67%	35,420	45%	31,327	88%	40%

Footnote: Age is based on the current age of the PLHIV as of the reporting period.

Care Cascade by Key Population

REGION	ESTIMATED PLHIV	DIAGNOSED PLHIV	1st 95) (Dx PLHIV/ Est. PLHIV)	ON ART	2nd 95 (On ART/ Dx PLHIV)	VL TESTED	VL TESTING COVERAGE (VL Tested/On ART)	VL SUPPRESSED	VL SUPPRESSION AMONG TESTED	3rd 95 (VL Suppressed/ On ART)
MALES HAVING SEX WITH MALES (MSM)	165,000	108,945	66%	75,534	69%	33,791	45%	28,691	88%	39%
PERSONS WHO INJECT DRUGS (PWID)	3,000	2,230	74%	568	25%	221	39%	211	90%	35%
OTHER MALES	29,300	12,673	43%	6,539	52%	2,988	46%	2,375	87%	40%
OTHER FEMALES	14,800	7,090	48%	3,087	44%	1,406	46%	1,170	87%	40%

Footnote:
Key Population: This group is identified based on their reported risky behaviors or exposures at the time of diagnosis. The classification focuses on the behaviors or exposures rather than the individual's sexual orientation, gender identity, or expression (SOGIE). KP tagging is among adult PLHIV (15+).
“Other Males” and “Other Females”: These refer to the general population of males and females who are not specifically categorized as part of the key population.